

Otitis media with effusion (OME), commonly known as glue ear, is the presence of fluid in the middle ear in the absence of infection. It can have a significant impact on children. This graphic summarises the recently updated NICE guidance, highlighting recent changes.

Orange boxes show changes between the new (2023) and previous (2008) NICE guidance

Guidance unchanged

* A normal-looking tympanic membrane does not exclude otitis media with effusion (OME).

* If suspecting, consider referral for formal assessment with tympanometry and hearing testing.

* Advise parents that parental smoking increases the risk of OME and breastfeeding reduces the risk, where this is relevant.

* Advise that there is no proven benefit from treatment with any medication, or any complementary or alternative therapies.

* Otorrhoea (ear discharge) — the most common complication occurring between 1 in 5 to 1 in 10 children: Consider a course of topical non-ototoxic antibiotics (such as ciprofloxacin) for 5-7 days for otorrhoea.

Concerns from parents/carers or professionals

Presentation

Be aware that children with OME often present with any of the following features

Guidance on presenting features is now split into two categories

Common features

Hearing difficulties
Delayed speech and language development
Ear discomfort Tinnitus

Also associated features

Behavioural problems; lack of concentration or attention
Poor educational progress
Balance difficulties Clumsiness

Formal assessment

If OME is clinically suspected on the basis of the child's clinical history and assessment of the presenting features, refer for further hearing assessment

Clinical examination, focus on:

Otoscopy Hearing testing Tympanometry
General development General upper respiratory health

OME with NO hearing loss
No treatment

OME associated hearing loss confirmed
Bilateral Unilateral

Consider co-existing causes of hearing loss

Sensorineural Non-organic Permanent conductive

Offer

Consider

Reassess hearing after 3 months

In children who are experiencing hearing difficulties that significantly affect day-to-day living, consider intervening earlier than the 3-month reassessment

Persistent bilateral hearing loss

Persistent unilateral hearing loss

Impacting daily living or communication?

No Yes

Hearing loss support strategies

All settings

Be close to and face the child when speaking to them
Minimise background noise
Use visual aids

In education

Inform staff of hearing loss related to OME
Ask if adjustments can be made in school eg. sitting near the front of class

Prepare the child for

Interventions
Ongoing management

Clinical management of hearing loss

Consider devices

Hearing aids
Auto-inflation
Bone conduction devices

Adenoidectomy
Not recommended in the absence of persistent and/or frequent upper respiratory tract symptoms

Update

When planning grommets for management of OME

Consider adjuvant adenoidectomy unless assessment indicates an abnormality with the palate

Do not offer

Antibiotics
Oral or nasal steroids
Decongestants
Homeopathy
Cranial osteopathy
Acupuncture

Surgical intervention

Insertion of temporary grommets or ventilation tubes to ventilate the middle ear is recommended

Ciprofloxacin; Consider a single dose given intraoperatively during grommet insertion

Water precautions; Keep ear dry, avoiding swimming, taking care when bathing or washing hair for 2 weeks after surgery

Reassessment; perform a postoperative hearing test 6 weeks after surgery for OME:

Hearing loss resolved; discharge and:
Consider follow up options

Hearing loss continues:
Investigate and manage appropriately

Refer to an ear, nose, and throat (ENT) specialist, if:

- 1- Hearing loss of any level is associated with a significant impact on the child's developmental, social, or educational status.
 - 2- Hearing loss requires urgent referral within 2 weeks to exclude additional sensorineural or other causes of deafness.
 - 3- The tympanic membrane is structurally abnormal (or there are other features suggesting an alternative Dx)
 - 4- There is a persistent, foul-smelling discharge suggestive of a possible cholesteatoma. Referral should be semi-urgent.
 - 5- The child has craniofacial anomalies, for example, Down syndrome or a cleft palate.
- Ö- If symptoms of otitis media with effusion (OME) recur, refer the child back to an ear, nose, and throat (ENT) specialist.